PLANNING BRIEF

# Long COVID burden and pilot planning brief

Deterministic synthesis of reviewed national burden, analyst-supplied planning assumptions, explicitly selected implementation context, evidence gaps, and interpretation guardrails.

**Brief provenance**

**National reviewed artifact:** meps-2024-long-covid-candidate-20260926T011343Z

**Generated:** 2026-10-15T02:41:56.328Z

## Planning insights summary

Deterministic planning preview — no live AI inference. Selected highlights; full reviewed findings, assumptions, provenance and selected requests follow in the Brief.

### Reviewed national evidence

**Mean annual all-cause medical spending per person: $14,238**

USD/person/year · 95% CI $10,668–$17,807

Universe: U.S. civilian noninstitutionalized population, adults age 18-64 · Denominator: Adults age 18-64 who met the operational current-Long-COVID definition at their Round 3/1 2024 interview, with valid TOTEXP24

meps24-gross-all-cause-spending-mean · meps-hc256-2024 · https://meps.ahrq.gov/mepsweb/data_stats/download_data_files_codebook.jsp?PUFId=H256&varName=TOTEXP24

**Annualized health care paperwork / navigation time: 24.5 hours/person/year**

hours/person/year · 95% CI 12.8–36.2 hours/person/year

Universe: U.S. civilian noninstitutionalized adult ESAQ respondent population · Denominator: Adults age 18-64 classified operational-current at the earlier Round 3/1 2024 interview, with later ESAQ administration in Panel 28 Round 5 / Panel 29 Round 3, ESAQELIG=1, positive ESAQWT24F, and valid EQPPAM53

meps24-healthcare-paperwork-time-mean · meps-hc256-2024 · HC-256 EQPPAM53; ESAQ time-cost documentation: https://meps.ahrq.gov/data_stats/download_data/pufs/h256/h256doc.shtml#2.5.9

**Mean K6 nonspecific psychological distress score: 5.9 score (0-24)**

score (0-24) · 95% CI 5–6.8 score (0-24)

Universe: U.S. civilian noninstitutionalized adult PSAQ respondent population · Denominator: Adults age 18-64 classified operational-current at the earlier Round 3/1 2024 anchor, with SAQELIG=1, positive SAQWT24F, and valid K6SUM42

meps24-k6-score-mean · meps-hc256-2024 · https://meps.ahrq.gov/mepsweb/data_stats/download_data_files_codebook.jsp?PUFId=H256&varName=K6SUM42

One finding per available domain; descriptive highlights, not a ranking of burden.

### Selected local implementation context

Reviewed West Virginia context is available. Select only the indicators relevant to your implementation decision.

## Analyst assumptions and deterministic planning outputs

Recognition → referral · Navigation support · illustrative-navigation-pilot-default · planning-assumptions-2026-09-v1

| | | | |
|---|---|---|---|
| Potentially eligible (assumption) | **12,000** | Intended reach (assumption) | **25% (0.25)** |
| Program capacity (assumption) | **3,000** | Baseline recognition window (assumption) | **12 months** |
| Planning recognition window (assumption) | **6 months** | Planning horizon (assumption) | **12 months** |
| Fixed program cost (assumption) | **$300,000** | Cost per participant (assumption) | **$300** |
| Participants within capacity (result) | **3,000** | Modeled capacity gap (result) | **0** |
| Person-month exposure difference (result) | **18,000** | Program-resource requirement (result) | **$1,200,000** |

Copied from the applied Pilot result. These are assumption-based planning quantities, not forecasts, causal effects, or savings.

## Interpretation boundaries

- Gross burden is not attributable burden.
- Local context informs implementation and does not modify national burden.
- Scenario outputs are deterministic planning quantities, not savings, ROI, causal benefit, or observed improvement.
- Out-of-pocket/self-family-paid spending is nested within total spending and must not be added to it.
- K6 measures nonspecific psychological distress and PHQ-2 is a depressive-symptom screen; neither is a psychiatric diagnosis.
- ESAQ time measures are AHRQ annualized hours-per-year measures; later ESAQ responses do not establish continued current Long COVID after the earlier Round 3/1 status anchor.

### Decision state

| | | | |
|---|---|---|---|
| **Burden estimates** | Reviewed national | **Scenario arithmetic** | Planning assumptions |
| **Evidence parameter** | No approved record loaded | **Local context** | Available, not selected |
| **Data requests** | None selected | | |

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## Reviewed national burden findings

## Economic / healthcare burden

**13 reviewed**

| Measure | Estimate | Confidence interval |
|---|---|---|
| **Represented population with operational current Long COVID** | 6.8M | 95% CI 5.9M–7.7M |
| **Gross annual all-cause medical spending** | $97.3B | 95% CI $72.0B–$122.5B |
| **Mean annual all-cause medical spending per person** | **Mean:** $14,238<br>**Typical Range (25th–75th percentile):** $1,116–$14,840<br>**Median:** $3,624 | 95% CI $10,668–$17,807 |
| **Out-of-pocket annual spending component** | $13.5B | 95% CI $6.5B–$20.5B |
| **Mean out-of-pocket annual spending per person** | $1,980 | 95% CI $945–$3,014 |
| **Mean annual office-based provider visits** | 12.4 visits/person/year | 95% CI 9.4–15.4 visits/person/year |
| **Proportion with at least one office-based provider visit** | 81.7% | 95% CI 77.2%–86.1% |

| Measure | Estimate | Confidence interval |
|---|---|---|
| **Mean annual outpatient department provider visits** | 2.3 visits/person/year | 95% CI 1.5–3.1 visits/person/year |
| **Proportion with at least one outpatient department provider visit** | 33.3% | 95% CI 27.9%–38.6% |
| **Mean annual emergency-room visits** | 0.4 visits/person/year | 95% CI 0.3–0.5 visits/person/year |
| **Proportion with at least one emergency-room visit** | 24.4% | 95% CI 19.7%–29% |
| **Mean annual hospital discharges** | 0.1 discharges/person/year | 95% CI 0.1–0.2 discharges/person/year |
| **Proportion with at least one hospital discharge** | 8.3% | 95% CI 5.5%–11.1% |

▼ **Interpretation and technical details**

**Domain limitations:** All spending and utilization estimates are gross/all-cause descriptive burden. The cohort is classified at the Round 3/1 2024 interview; annual 2024 spending/utilization may occur before or after that point and does not imply year-long symptom persistence. Out-of-pocket spending is nested within total spending and is not additive.

- **Represented population with operational current Long COVID:** Survey-weighted number of status-classifiable U.S. civilian noninstitutionalized adults age 18-64 meeting the documented operational Round 3/1 2024 current-Long-COVID rule.

  System boundary: National descriptive population estimate
- **Gross annual all-cause medical spending:** Calendar-year 2024 gross all-cause health care spending among adults age 18-64 who met the operational current-Long-COVID definition at their Round 3/1 interview in 2024.

  System boundary: National all-cause health care expenditures
- **Mean annual all-cause medical spending per person:** Mean calendar-year 2024 all-cause health care spending among adults age 18-64 who met the operational current-Long-COVID definition at their Round 3/1 interview in 2024.

  System boundary: National all-cause health care expenditures
- **Out-of-pocket annual spending component:** Self/family-paid component of calendar-year 2024 all-cause spending among adults classified operational-current at the Round 3/1 2024 interview.

  System boundary: National all-cause health care expenditures; source-of-payment component
- **Mean out-of-pocket annual spending per person:** Mean self/family-paid component of calendar-year 2024 spending among adults classified operational-current at the Round 3/1 2024 interview.

  System boundary: National all-cause health care expenditures; source-of-payment component

- **Mean annual office-based provider visits:** Mean calendar-year 2024 all-cause office-based provider visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one office-based provider visit:** Share with at least one office-based provider visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual outpatient department provider visits:** Mean calendar-year 2024 all-cause outpatient department provider visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one outpatient department provider visit:** Share with at least one outpatient department provider visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual emergency-room visits:** Mean calendar-year 2024 all-cause emergency-room visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one emergency-room visit:** Share with at least one emergency-room visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual hospital discharges:** Mean calendar-year 2024 all-cause hospital discharges among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one hospital discharge:** Share with at least one hospital discharge during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization

## Healthcare-work / administrative burden

4 reviewed

| Measure | Estimate | Confidence interval |
|---|---|---|
| **Annualized time on own medical visits / filling prescriptions** | 76.5 hours/person/year | 95% CI 45.7–107.2 hours/person/year |
| **Annualized time managing medical bills / insurance** | 27.5 hours/person/year | 95% CI 13.5–41.5 hours/person/year |
| **Annualized health care paperwork / navigation time** | **Mean:** 24.5 hours/person/year<br>**Typical Range (25th–75th percentile):** 0 hours/person/year–12 hours/person/year<br>**Median:** 3 hours/person/year | 95% CI 12.8–36.2 hours/person/year |
| **Prior approval denied or delayed — yes/no response universe** | 34.3% | 95% CI 27.2%–41.5% |

▼ **Interpretation and technical details**

**Domain limitations:** ESAQ native time domains are reported separately and are not summed. ESAQ respondents were classified operational-current at the earlier Round 3/1 2024 anchor; later ESAQ responses do not prove continued current status. ESAQ time measures are top-coded by AHRQ and have item-response limitations.

- **Annualized time on own medical visits / filling prescriptions:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for own medical visits and filling prescriptions among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Annualized time managing medical bills / insurance:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for paying or managing medical bills and insurance among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Annualized health care paperwork / navigation time:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for paperwork, finding doctors, plan information, and approvals among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Prior approval denied or delayed — yes/no response universe:** Proportion reporting that health insurance denied or delayed prior approval among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 previously classified operational-current at the earlier Round 3/1 2024 interview who supplied a substantive Yes/No response.

  System boundary: National ESAQ prior-approval experience

## Mental-health / human burden

4 reviewed

| Measure | Estimate | Confidence interval |
|---|---|---|
| **Mean K6 nonspecific psychological distress score** | 5.9 score (0-24) | 95% CI 5–6.8 score (0-24) |
| **Serious nonspecific psychological distress (K6 >=13)** | 14.1% | 95% CI 9.3%–18.8% |
| **Mean PHQ-2 depressive-symptom screen score** | 1.3 score (0-6) | 95% CI 1.1–1.6 score (0-6) |
| **PHQ-2 screen-positive (score >=3)** | 20.6% | 95% CI 14.7%–26.4% |

▼ **Interpretation and technical details**

**Domain limitations:** K6 measures nonspecific psychological distress; PHQ-2 is a depressive-symptom screen. PSAQ respondents were classified operational-current at the earlier Round 3/1 2024 anchor; later PSAQ outcomes do not prove continued current status. Neither is a psychiatric diagnosis, monetized burden, or causal Long COVID effect.

- **Mean K6 nonspecific psychological distress score:** Mean official K6 nonspecific psychological distress score among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ nonspecific psychological distress
- **Serious nonspecific psychological distress (K6 >=13):** Proportion meeting AHRQ's published K6 >=13 serious-distress category among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ nonspecific psychological distress
- **Mean PHQ-2 depressive-symptom screen score:** Mean official PHQ-2 depressive-symptom screening score among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ depressive-symptom screening
- **PHQ-2 screen-positive (score >=3):** Proportion with PHQ-2 score >=3 among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ depressive-symptom screening

## Pilot planning assumptions

**Illustrative planning defaults — edit before use** These inputs are planning assumptions, not observed evidence.

| | | | |
|---|---|---|---|
| Potentially eligible | **12,000** | Intended reach | **25% (0.25)** |
| Program capacity | **3,000** | Baseline recognition window | **12 months** |
| Planning recognition window | **6 months** | Planning horizon | **12 months** |
| Fixed program cost | **$300,000** | Cost per participant | **$300** |

MODELED PATHWAY STEP
**Recognition → referral**

ASSUMED MECHANISM
**Navigation support**

Reach, capacity, recognition-window assumptions, and program-resource inputs apply only within this modeled step and mechanism.

**Outside this model:** Reviewed national burden estimates; Unrelated care-pathway stages; Treatment effectiveness; Clinical outcomes.

## Scenario outputs

Deterministic planning quantities from the applied assumptions above; these are not a forecast or observed evidence.

| | | | |
|---|---|---|---|
| Participants within capacity | **3,000** | Person-month planning exposure difference | **18,000** |
| Program-resource requirement | **$1,200,000** | | |

### Decision trace

Evidence → explicit analyst context → planning scope → evidence/data gap. This is a provenance and decision ledger, not a proven causal chain.

**01** **Released observed burden**
Reviewed national MEPS burden · 21 reviewed estimates · national descriptive evidence

02 **Explicitly selected reviewed local constraint/context**

No explicit local constraint selected

03 **Target pathway step**

Recognition → referral

04 **Planning mechanism**

Navigation support

05 **Measurement / data request needed**

No current operational data request attached

06 **Supported claim boundary**

No reviewed intervention-effect parameter is available. The scenario remains assumption-based; causal benefit, savings, ROI, and preventable-cost claims remain unavailable.

## Local implementation context

**Reviewed West Virginia state context is available, but no local context has been selected for this brief or used in Pilot planning.**

Review the available state evidence on the Local Context page. A record enters this Brief only through an explicit Include-in-Brief selection or an explicit Use-in-Pilot selection.

## Intervention-effect evidence

**No reviewed intervention-effect parameter is loaded.**

Causal benefit, savings, ROI, and preventable-cost claims remain unavailable.

## What to collect next

**No data request has been created or loaded.**

Data requests are planning artifacts, not observed evidence. Add or select them explicitly before export.